Medical Paediatric Virtual Ward

Referral information

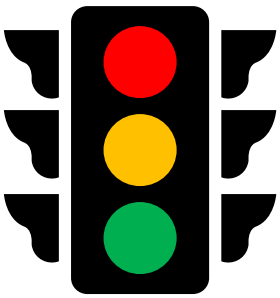
General Information

- The Virtual Ward can be considered for any child that would otherwise require an acute inpatient stay.
- Care is tailored to the child and includes video calls, home visits and face to face reviews.

Primary Criteria

☑ Reviewed by a medical paediatric consultant
☑ Clear treatment goal with defined end point.
☑ Clinically stable
☑ No active social concerns
☑ Family consenting
☑ Postcode covered by GGC acute services *

Patient selection *(Must first meet the primary criteria above)*



Red = Not suitable for the Virtual Ward
Fever without a focus, acute sepsis concerns, oxygen requirement (above baseline), IVF requirement. Consider step-down to virtual ward once stable

Amber = Ideal patients for the Virtual Ward
Borderline clinical status and/or needing further treatment or monitoring. See the Amber boxes below for example scenarios. These lists are not exhaustive.

Green = Does not require the Virtual Ward
Children that would otherwise have been discharged. Routine outpatient care. Scenarios usually managed by primary care, CCNs or health visitors.

Support & Monitoring Pathways
(Includes clinical observations)
**Must be within a 15 mile radius of the RHC for a monitoring pathway*

Clinical monitoring (e.g.)

☑ Acute respiratory illnesses (e.g. bronchiolitis, URTI) with borderline status
☑ Children on oral antibiotics needing further review (e.g. cellulitis, LRTI).
☑ Complex patients with intercurrent illnesses
☑ Stable children >24h into IV Abx awaiting culture results .
☑ Step down care in severe conditions that have stabilised (e.g. IV therapy for wheeze)
☑ Hydration monitoring (includes gastroenteritis, tonsillitis)

Clinical Support Pathways
(Clinical observations not required)

Investigations (e.g.)

☑ Bloody diarrhoea follow up
☑ Malaria screening follow on tests
☑ Blood test monitoring
☑ Urinalysis (Proteinuria, haematuria)
☑ Stable and awaiting inpatient imaging

Medication administration (e.g.)

☑ NG
☑ Rectal
☑ IM/SC/IV (max daily only)

Condition specifi (e.g.)

☑ Faecal impaction
☑ Infant feeding/NG feeding in older children
☑ Incidental hyperglycaemia

To refer contact the Virtual Ward Team:
8am – 8pm 0141 452 (8)4467